

Q Are stretch marks inevitable? Why do we get them and is there anything I can do?

Stretch marks (or striae gravidarum) can appear on your abdomen, thighs, hips, arms, breasts, and buttocks. They tend to run in families, so if your mother, grandmother, or sisters had them, it's likely that you will too. There's very little you can do to prevent them, but you're not alone since 90 percent of women get them.

They're caused when the collagen—the connective tissue in your skin—weakens, stretches, and tears as your belly grows. Hormonal changes make the problem worse.

A diet that's rich in skin-supporting nutrients (found in nuts and seeds, and fresh fruits and vegetables, which contain high levels of antioxidants), and not “eating for two” (see p.59) can help to minimize the effects. Try to gain weight steadily throughout your pregnancy (see chart, p.126) so that your skin has time to stretch. If you stay fit you may be able to limit

the appearance of stretch marks. Keeping your skin supple with moisturizing creams will help it to ping back into shape after you've had your baby, but studies show that they have limited effect on stretch marks. This is because collagen works deep within the layers of your skin, so creams you put on top may not be enough.

Once you've had your baby, the stretch marks will gradually fade over time. In most women they will disappear altogether, or remain only as fine, silvery lines by the time a baby is six months old.

Q How common is it for skin pigmentation changes to occur and what exactly is the “butterfly mask?”

Chloasma, also known as the “butterfly mask” or melasma, gives you dark or pinkish patches over your cheeks, nose, eyes, and forehead—sometimes in a butterfly shape, hence the common name.

Butterfly effect Women with darker skin may be more susceptible than women with lighter skin.



Pregnant women are prone to this because the increased levels of estrogen in their bodies make more melanin—the pigment that gives your skin, hair, and eyes their color, and which protects your skin from the sun's harmful UV rays. Chloasma affects around half of all pregnant women, and is more common in those with darker skin, whose levels of melanin are already high. This side effect is completely harmless to you or your baby's physical health, but some women can be upset by it.

Being in the sun can make it worse, so stay in the shade and use a high-SPF, broad-spectrum sunscreen that does not contain oxybenzone. You're probably already taking folic acid (see p.49), but if you aren't, or if you're past the 12 weeks' mark and have stopped taking it, talk to your doctor about a daily supplement; some studies show that being low in the mineral folate can make you more susceptible to chloasma. Finally, remember that it is almost always temporary; your skin should return to normal not long after you've given birth.

Cover up Exposure to the sun can make the mask more pronounced. Wear a hat to protect your face.

DARKENING SKIN

Increased levels of melanin in your skin are responsible for all kinds of skin changes that may occur in pregnancy. These can include:

» **A greater number of freckles** on your face.

» **Darkening in moles, birthmarks, and scars.**

» **The line that may appear** from your navel to your pubic bone, known as the “linea nigra.”

» **Your ability to tan** more easily.

» **Darker areolae** around your nipples.



Linea nigra This is situated where the left and right vertical abdominal muscles meet. Your growing belly causes them to separate and darken.

